



Boundary spanners: Negotiating connections across primary care and domestic violence and abuse services

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ABSTRACT

Improving access to support for people experiencing domestic violence and abuse requires better connections between healthcare services and specialist domestic violence and abuse (DVA) support agencies. We examined the work involved in restructuring the relationship between primary care and specialist DVA support services. This was part of a broader study of the implementation of a general practice DVA training and support programme (IRIS). We conducted an ethnography in two different UK areas where the IRIS programme was being delivered. We investigated the work done by specialist DVA workers (Advocate Educators) in the dual role of providing training to GPs and advocacy support to patients. Drawing on concepts of boundary actors and boundary objects, we examined how interactions between clinicians and patients changed after the introduction of the IRIS programme. The referral pathway emerged as a boundary object, meeting a shared ambition of general practitioners and patients to distribute responsibility for addressing DVA. However, maintaining this as a boundary object-in-use required significant, and often unseen, work on the part of the Advocate Educator as boundary spanner. Our study contributes to scholarship on boundary work by highlighting the role of marginal boundary actors in maintaining the use of boundary objects among disparate groups.

1. Introduction

There is a growing emphasis on improving patient care by building better pathways between different parts of the health care system in the UK. This includes improving links within healthcare services, between different public services, between National Health Service (NHS) and health research communities, and between public and community services. However, studies of collaboration demonstrate clearly that enabling structural connections between different types of organisations and groups does not in itself guarantee improved quality of interaction (Rycroft-Malone et al., 2015).

Wenger (1998) proposed that different groups of professionals operate in different communities of practice. What is required to build connections are practices or processes that have symbolic resonance in different worlds and enable interaction between them (Melville-Richards et al., 2019) and people operating in bridging positions at the boundaries between groups (Levina and Vaast, 2005). These are often referred to respectively as boundary objects (Star and Griesemer, 1989) and boundary spanners (Long et al., 2013).

This paper explores the boundary work involved in improving connections between primary healthcare services in the UK and

specialist domestic violence and abuse (DVA) support services. We investigate the work done by boundary spanners in the dual role of providing training to GPs and advocacy support to patients.

2. Background

2.1. The identification and referral to Improve Safety (IRIS) programme

This paper takes as a case study the implementation of a primary care DVA intervention developed in the UK called Identification and Referral to Improve Safety (IRIS). The intervention aims to improve identification of DVA in primary care settings and increase referral to support services. In the UK, DVA is defined as ‘any incident or pattern of incidents of controlling, coercive, threatening behaviour, violence or abuse between those aged 16 or over who are, or have been, intimate partners or family members regardless of gender or sexuality’ (Home Office, 2016). Abuse can encompass, but is not limited to, psychological, physical, sexual, financial, and emotional abuse.

The IRIS programme includes: training for the whole team co-delivered by a specialist DVA support worker (Advocate Educator) and a clinician; a direct referral pathway to the Advocate Educator (AE)

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